

Who Watches the Watchmen?

On loss aversion, status quo bias, and the cognitive failures shaping America's GLP-1 coverage gap.

When I was introduced to Economics via the construct of humanity's underlying rationality, I was overjoyed. In front of me was a subject that explained the *physics* of the world so well! It seemed I could solve so many mysteries around me. This was Grade 9 introductory econ – and naturally, as I progressed onto an undergrad in Economics and more generally came to make sense of the world and everything in it, I realized the naivety of that perspective.

Since about four decades before this introductory economics class I refer to, academics like Amos Tversky and Daniel Kahneman have motivated that we aren't indeed the rational agents¹ we have been presuming we are. We discount the future, we fear losses more than we cheer gains, and default to whatever is the laziest option. More recently, the economist from Chicago, Richard Thaler connected these psychological traits to everyday human economics. He popularized the concept of *nudging* individual agents to make the right choices for themselves, with Barrack Obama's first chief-of-staff Cass Sunstein, in a book called 'Nudge.'²

Governments, persuaded by this scholarship, have built institutions around this idea – where individuals like you and me, for lack of better control of ourselves, are nudged along our day-to-day activities.³ Restaurants attach a calorie count to my order, the driver license application presumes my agreement for organ donation and my electricity bill smiles at me. *Nudge units* and *behavioural insights teams* have propped up across the world. The premise being, if we understand the bias, we can design around it.

This essay flips that equation and audits the bias in these policy designers themselves.

Glucagon-like-peptide-1 agonists (GLP-1s), or which we know generally as Ozempic, Mounjaro, Wegovy or Zepbound depending on who you buy from, are arguably the most rigorously validated pharmacological interventions against obesity in our history. The SELECT trial enrolled 17,600 patients across 41 countries and demonstrated a 20% reduction⁴ in major cardiovascular events in people with obesity who do not have diabetes. The World Health Organization added GLP-1 therapies to its Essential Medicines List in 2025.⁵ One of medicine's highest honours, the Lasker Award, went to the scientists who discovered the underlying hormone. The evidence, in short, isn't contested. The policy response for its access however, is.

As of early 2026, Medicare in the United States does not cover GLP-1s for obesity alone. The Trump administration declined to finalise the Biden-era coverage proposal in April 2025, citing only that the expansion was "not appropriate at this time." More than two thirds of Medicare beneficiaries are classified as overweight or obese. The drug works and the patients exist. The coverage to connect them is lacking.

¹ Kahneman, D., & Tversky, A. (1979). Prospect theory: An analysis of decision under risk. *Econometrica*, 47(2), 263–291. <https://doi.org/10.2307/1914185>

² Thaler, R. H., & Sunstein, C. R. (2008). *Nudge: Improving decisions about health, wealth, and happiness*. Yale University Press.

³ Weatherby, L. (2023, November 30). Opinion | A Few of the Ideas About How to Fix Human Behavior Rest on Some Pretty Shaky Science. *The New York Times*. <https://www.nytimes.com/2023/11/30/opinion/human-behavior-nudge.html>

⁴ Lincoff, A. M., Brown-Frandsen, K., Colhoun, H. M., et al. (2023). Semaglutide and cardiovascular outcomes in obesity without diabetes. *New England Journal of Medicine*, 389(24), 2221–2232. <https://doi.org/10.1056/NEJMoa2307563>

⁵ World Health Organization. (2025, December 1). WHO issues global guideline on the use of GLP-1 medicines in treating obesity. <https://www.who.int/news/item/01-12-2025-who-issues-global-guideline-on-the-use-of-glp-1-medicines-in-treating-obesity>

This essay isn't about medical uncertainty. It is not even, primarily, about cost. It is about a cognitive bias, operating not in patients, but in policymakers. The same loss aversion, status quo bias, and affect heuristic that behavioural economists have catalogued in citizens for decades are shaping institutional decisions of the very governments that claim to have corrected for them. And unlike the citizen nudged toward a better pension default or a healthier cafeteria option, nobody is nudging the nudge-*rs*.

What are GLP-1 agonists, and how do they work?

GLP-1, by itself, is a hormone that the body already produces. It is secreted by L-cells in the intestine in response to eating, and by neurons in the brainstem. Under normal conditions, it signals the pancreas to release insulin, slows gastric emptying so food moves through you more gradually, and crucially, communicates satiety to the brain. People with obesity frequently have blunted GLP-1 response – and its signal is weaker, arrives later and is less effective at telling the brain that enough has been eaten.

GLP-1 receptor agonists are synthetic analogues of this hormone, engineered to replicate and sustain its effects at pharmacological doses. They act on the gut, slowing gastric emptying and promoting satiety after smaller meals. They also act on the pancreas, improving glucose regulation – which is how their initial approval as a counter to diabetes came to be. Outside of this very heavy medicine-speak, Huberman's Lab explains, these importantly also act on the brain.⁶ When GLP-1 receptors in the brain are activated, dopamine release decreases and so the reward signal for eating reduces. Patients on these drugs frequently report losses in appetite, and research currently concludes that this effect is neurobiological.

Three agents are currently FDA-approved for chronic weight management. Liraglutide (Saxenda), approved in 2014, produces average body weight reductions of around 5–8%. Semaglutide (Wegovy), approved in 2021, achieves approximately 15%. Tirzepatide (Zepbound), approved in 2023 as a dual GIP and GLP-1 receptor agonist, reaches 20–22% in clinical trials, percentages previously achievable only through surgical interventions. Prescriptions for this drug class increased 300% between 2018 and 2023, and the global market for it was valued at USD 53 billion in 2024.

The SELECT trial is the proof point that changes the policy conversation entirely. Its findings: weekly semaglutide at 2.4mg reduces the incidence of cardiovascular death, non-fatal heart attack, and non-fatal stroke by 20%, over a period of 40 months. This is the first instance any medication or lifestyle therapy has been proven to reduce cardiovascular events in adults with obesity who do not have diabetes. Given the significance of the drug's effectiveness, the policy conversations around the drugs coverage in insurance are worth examining.

Loss aversion in policymakers

The most straightforward explanation for the Medicare coverage gap is also the most behaviourally honest one. It is expensive to say yes, and cheap to say nothing.

The Congressional Budget Office estimated⁷ in 2024 that expanding Medicare Part D coverage to cover GLP-1s for obesity would cost the federal government USD 35.5 billion between 2025 and 2034. This large, visible and attributable number sits significantly on a budget line and therefore has a political constituency. Comparatively, statistical patients and their potential savings by

⁶ Huberman Lab Clips. (2024, September 11). How Do Ozempic, Mounjaro & Other GLP-1 Agonists Work? | Dr. Zachary Knight & Dr. Andrew Huberman. YouTube. <https://www.youtube.com/watch?v=zRXC2pEbj5w>

⁷ Congressional Budget Office. (2024, October). How would authorizing Medicare to cover anti-obesity medications affect the federal budget? <https://www.cbo.gov/publication/60816>

virtue of avoiding accumulated cardiovascular disease over the following decade, have no budget line mention, no face and no constituency. The USC Schaeffer Center estimated⁸ these statistical potential benefits to be between USD 176 billion and USD 245 billion over the same ten-year window.

This is loss aversion operating at institutional scale. Kahneman and Tversky demonstrated how losses feel twice as painful as equivalent gains feel pleasurable. The certain present cost outweighs an uncertain future saving, even when the arithmetic clearly favours action. The policymaker weighing USD 35.5 billion in drug spend against USD 245 billion in downstream savings isn't making a conservative decision, but a cognitively biased one.

What makes this particularly legible as loss aversion, rather than ordinary fiscal caution, is the asymmetry in what Medicare already covers. GLP-1s are covered when prescribed for type 2 diabetes. They have been covered for cardiovascular risk reduction as well, citing the same SELECT data. The molecule, the injection and the cardiovascular benefit are identical. What is different is the diagnostic label, and with it the political optics of the spend. Covering a diabetes drug feels like treating a disease, but covering an obesity drug feels like paying for a shortcut. The timeline of approvals by FDA indicate this too: the SELECT data okayed cardiovascular indication for Wegovy in March 2024. Since late 2024 then, a proposal to expand Medicare coverage of GLP-1s for obesity has been just that – a proposal. The political calculus of a visible budget line that comes with a USD 35 billion drug expansion and resultant rises in premiums is of course load-bearing for today's policymaker. Loss aversion is a deeply human reaction to the uncertainty and accountability they own. The question is why governments happily apply this logic to citizens' pension funds but not their own drug coverage decisions.

Misaligned incentives & the status quo

Status quo bias, Samuelson and Zeckhauser show,⁹ is the tendency to prefer the current state of affairs over alternatives, even when the evidence favours switching. Departures from the reference point are evaluated as losses. The interventions for obesity have centred around lifestyle-choices for three decades.

Let us consider what switching then would require. A government that moves from "lifestyle intervention-first" to "medication access for obesity" must make an active, visible and politically attributable choice. It must defend against inevitable headlines about the government paying for slim bodies. The accountability asymmetry makes it worse. If an individual-focussed intervention (lifestyle choice nudges) fails, the individual is to blame. But, if a cost expansion (by increasing drug coverage) shows negative eventualities, the policymaker must own it. In institutional settings, while status quo bias is a simple preference for inertia, it is also a rational response to an accountability structure that punishes switching and forgives staying still.

For example, Denmark's 2011 fat-tax was repealed just 15-months after introduction, under pressure from the food industry and its citizenry finding it administratively cumbersome. The evidence was not the reason it was repealed. The political cost of maintaining a disruptive direction from the reference point proved too high to sustain.

⁸ Doshi, J. A., Patel, A., & Rubinstein, E. (2024). Long-term savings from GLP-1 coverage in Medicare: A cost-offset analysis. USC Schaeffer Center for Health Policy and Economics. <https://doi.org/10.1001/jamahealthforum.2025.0905>

⁹ Samuelson, W., & Zeckhauser, R. (1988). Status quo bias in decision making. *Journal of Risk and Uncertainty*, 1(1), 7–59. <https://doi.org/10.1007/BF00055564>

The United Kingdom's National Health Service (NHS) offers a contrasting example. It *began* commissioning Wegovy for weight management in 2023, with phased rollouts.¹⁰ The decision was grounded in clinical criteria, structured around a tiered eligibility and framed explicitly as treatment for a chronic disease. What matters analytically is the framing decision, not the execution. By categorising obesity as a chronic condition requiring medical management, the NHS steered to a new reference point to frame a renewed outlook to obesity in society.

The United States has not made that framing decision. Without one, every proposal to expand GLP-1 coverage begins from a position of defending a departure, as against defending reversing an omission. Status quo bias does not require bad faith. It does not require ignorance of the evidence. It requires only that the costs of switching feel more immediate and attributable than the costs of staying still – and in the American political economy of drug coverage, they reliably do.

Afflicted with the affect heuristic

The affect heuristic, as Paul Slovic and his colleagues demonstrated,¹¹ describes how emotional responses to a subject contaminate the rational assessment of its costs and benefits. When something triggers a negative feeling, we simultaneously overestimate its risks and underestimate its benefits. Ozempic triggers a very specific feeling in public and political discourse. It is a shortcut. It is cheating. It is the government subsidising personal irresponsibility, and maybe even vanity. Language like, “we don’t want to create dependency,” “patients should lose weight naturally” surfaces often in policy debates. This isn’t evidence assessment, but moral discomfort.

The ‘moral hazard’ argument against GLP-1 coverage in insurance argues that drug accessibility will encourage disregard for all weight-management. Clinical trial data shows that patients increase physical activity as weight drops. Ideally prescription standards also motivate the right usage with gradual dosing and simultaneous protein intake and resiliency training. This is not unique to GLP-1. It is a pattern seen back in 1990s, when HIV antiretroviral treatment faced similar resistance. There were concerns that treatments would reduce safe behaviour and that governments shouldn’t subsidise consequences of choice. The reframing that eventually shifted policy was cultural, and a critical mass shifted to thinking about it as a disease rather than a moral failure. This requires a different kind of policy work entirely.

What makes this particularly costly is that the affect heuristic operates upstream of the other two biases. Loss aversion and status quo bias make policymakers reluctant to act on evidence they broadly accept. The affect heuristic makes them reluctant to accept the evidence in the first place. The moral discomfort of GLP-1s as a category of intervention means that the drug’s cardiovascular outcomes data is processed through filters of suspicion, rather than independently. They are perceived *not sufficient*, in a way that the same findings are felt *sufficient* for a diabetes drug. The difference is not in the data, but in the emotional prior.

Nobody here is necessarily acting in bad faith. The motivation of this essay is to highlight that loss aversion, status quo bias and affect heuristic aren’t moral failures but are cognitive ones – which we know are predictable, well documented and in the right institutional environments,

¹⁰ DHSC Media Team. (2023, September 4). Accessing Wegovy for weight loss: Everything you need to know - Department of Health and Social Care Media Centre. healthmedia.blog.gov.uk.

¹¹ Slovic, P., Finucane, M. L., Peters, E., & MacGregor, D. G. (2002). The affect heuristic. In T. Gilovich, D. Griffin, & D.

Kahneman (Eds.), *Heuristics and biases: The psychology of intuitive judgment* (pp. 397–420). Cambridge University Press. doi.org/10.1017/cbo9780511808098.025

also correctable. The question this essay has been building towards is what that environment should look like, and that is the work of the conclusion.

Nudging the nudge-ers

Loss aversion made the cost of saying yes feel larger than the benefit of saying yes, even when the arithmetic disagreed. The correction then, could be to make future savings as visible as the present spend. To require that coverage decision model downstream health-cost offsets (savings) over a ten-year horizon, not just the year-one drug budget. Not new for behavioural economics, the same logic is recommended for individual retirement savings decisions: make the future concrete because humans systematically discount what they cannot see. The same principle applies to the status quo problem. If obesity is formally reframed as a chronic disease requiring medical management at the federal level, like NHS did in 2023, coverage becomes a default rather than a departure. The omission becomes the thing that requires justification, not the intervention. Both corrections are doing the same work: making the cost of inaction as legible as the cost of action.

The affect heuristic made the drug feel like a shortcut before the data was assessed, and filtered the data through that feeling after. To correct for affect heuristic is harder because sub-culture doesn't update at will. Possibly, structured pre-mortem requirement could make some progress. This mandated adversarial review would ask whether the decision would survive scrutiny from a bias-aware assessor. At minimum, this coerces the emotional prior into the open, where it can be examined. The Financial Conduct Authority in the United Kingdom already uses behavioural audit frameworks of this kind for financial regulation.¹² The State Auditor General and the dissenting opinion in judgements play similar roles. There is no foreseeable reason drug coverage decisions should be exempt.

Neither intervention is radical. Both of them are applications of existing tools to a domain that has so far been exempted from them. Governments have spent decades nudging citizens toward better decisions. It is time to return the favour.

¹² Financial Conduct Authority. (2023). Behavioural economics and regulatory decision-making: A framework for bias-aware policy review. FCA Occasional Paper. <https://www.fca.org.uk/publication/occasional-papers/occasional-paper-1.pdf>